INTERVIEWER NOTE · NOT PART OF DANA'S PACKAGE

Behind the Design: WN7 Recovery Strategy

An LxD decision-making and reflection note

This note explains how I approached the Northstar WN7 case: how I reframed the request, the five decisions that shaped the proposal, how I worked with my coding-agent harness, and what remains uncertain.

01 From request to performance problem

Dana asked for a five-minute video to help Guides handle upset callers without losing control of the conversation, and she needed something practical for supervisors within days. I took the request seriously, and that meant understanding what would actually produce the change she wanted. Her own framing pointed the same way. On September 5 she said she could speak to Learning "once we know whether there is a behavior we can change," and her request noted that she hadn't "dug into the data yet." Elias, in Quality, asked Learning to say what would be different on a call if the recommendation worked, because a completion count wouldn't answer that.

So I treated the video as one possible solution and investigated before choosing a format: the Orbit call sample and operations data, meeting notes and threads, Guide and Lead feedback, and the recovery guidance itself.

The data pointed somewhere specific. In a systematic sample of 204 eligible General Care calls from September 21–30, **115 (56%) went through generic troubleshooting** instead of recovery. Of the 159 with a serial check recorded, **70 still ended in generic troubleshooting**.

While that established a performance gap between identifying an eligible customer and completing the recovery, it didn't establish the cause. A recorded check doesn't show what a Guide saw or understood, and the qualitative sources pointed several ways at once: unclear Plus answers, an Orbit result some Guides couldn't read, uneven access, inconsistent Lead messaging.

> "I hear a search problem, an eligibility problem, a conversation problem, and a completion problem."
>
> Priya Bell, Manager of General Care · September 18 shift feedback

Keeping the gap and its possible causes separate shaped everything after it.

QUANTITATIVE	QUALITATIVE	POLICY AUTHORITY	OPERATIONAL CONSTRAINTS
Orbit call sample, Sep 21–30 (204 eligible calls); Orbit operations data	Sep 18 shift feedback; Sep 22 Guide comments; Sep 24 listening notes; Lead requests in Quality's team notes	BH-4419 and the PO-117 care desk bulletin; Sep 10 Product and Care approval call	Sep 19 workforce planning; Sep 21 care systems sync; Oct 1 CX check-in

02 Five key decisions that shaped the strategy

These decisions emerged at different points as the evidence, design, and planning developed.

1 Align Leads before expanding to Guides

DECISION Make Lead alignment the foundation, with Guide support built on top of it.

WHY Guides go to their Leads mid-call, and the release had already relied on Leads to pass the

change along without equipping them: two Leads said their Guides first heard about BH-4419 from a reviewer. If Leads give different answers, Guide training can't produce consistent calls.

WHAT CHANGED The first resource became a 25-minute working meeting built on two cases, with two one-page references Leads keep. The rollout aligns each Lead before their own Guides start, and the Lead's role moved from "pass this on" to diagnosing and coaching against one standard.

2 Address performance, not just knowledge

DECISION Treat the gap as a performance problem with learning and non-learning causes.

WHY A Lead put it plainly: "People can tell you all five CLEAR steps in a quiz. The issue is the pivot from listening to acting." Several barriers weren't about knowledge at all: no approved answer for a pending Plus credit, an Orbit result some Guides couldn't locate, possible permission gaps. A video can communicate the process but can't fix those.

WHAT CHANGED The package pairs learning with in-the-moment support (the Guide Call Path) and makes operational validation part of the plan, naming who must confirm Plus guidance, Orbit access and delivery before any Guide sees it. The video wasn't discarded: the plan keeps a narrower Orbit tool walkthrough only as an optional item, not a launch prerequisite, and only if an approved screen reference and General Care access are confirmed.

3 Differentiate support by audience

DECISION Design around what each audience has to do, not one course for everyone.

WHY Leads and Guides kept asking for different things. Priya acknowledged, "We have not told Leads what to coach," and Leads asked for cues and clips. A Guide asked to know "what I click and what I say when the customer asks whether we are replacing it." So Leads need to recognize good execution, diagnose a breakdown before coaching it, and tell a performance gap from an operational blocker. Guides need the next step at the moment of decision and a chance to practice the hard call before it counts.

WHAT CHANGED Leads get the meeting, a Quick Reference and Coaching Cues. Guides get the Call Path and a short individual practice, connected through the huddles Leads already run with one Lead-facilitated slide. The practice, *Make the Call*, centers on a **Not eligible** result: the symptoms fit the WN7 pattern but the serial doesn't qualify, so the Guide has to make and explain the decision without promising an unsupported recovery. A Guide asked for exactly that: "A practice case with an ineligible unit would help, because the easy example always has a happy ending."

How it evolved: I first proposed a short Guide practice huddle. It became an individual digital scenario, which kept practice out of an extra live session while the team huddle and slide stayed as the place Leads introduce the change.

4 Minimize operational disruption

DECISION Treat time off the phones as a design constraint, not an afterthought.

WHY There are roughly 900 hourly Guides on rotating schedules, Specialist Care can absorb only a limited number of WN7 transfers, and Workforce warned that a broad session would push abandonment higher.

WHAT CHANGED Every component is small and fits existing routines: a 25-minute Lead meeting, reinforcement in existing huddles, a 5–7 minute individual practice in scheduled work time (subject to Workforce scheduling), and a one-page Call Path for live calls. The aim was less disruption without thinner support.

How it evolved: GPT first proposed a 25-minute Lead meeting. I cut it to 20 because Leads have little spare time, then restored 25 to give the case discussion and alignment room, accepting the cost in Lead time on the grounds that one session replaces fragmented coaching later. The agenda settled at 3 · 9 · 10 · 3, with the middle 19 minutes run as one exercise.

5 Pilot, evaluate, then scale

DECISION Start with two or three representative Lead teams, subject to Workforce approval, and expand by wave only on evidence.

WHY The intervention depends on operational conditions that aren't confirmed yet, and completion data alone wouldn't show whether calls changed. Marisol said she needed "a reason to believe it will work."

WHAT CHANGED Five phases, each ending in a natural checkpoint to regroup, review the emerging evidence, make adjustments and decide whether to proceed. Expansion needs Care leadership's judgment and Workforce's approval for each wave, and delivery is reported as reach, never as proficiency. Fewer avoidable escalations to Specialists is a hoped-for benefit, not a demonstrated outcome.

Learner-facing samples to look at first: the **Guide Call Path** (performance support at the point of need), then **Make the Call** (how that extends into practice). The Huddle Sequence Slide shows the Lead-to-Guide connection.

These five decisions had the greatest influence on the overall strategy, but they represent only part of the design process. Additional decisions involving scenario selection, coaching structure, resource formats, operational constraints, and implementation details are documented in the accompanying **Design Decision Log**.

03 How I worked with the coding-agent harness

I approached this project with AI as a collaborative development environment, not simply a way to generate finished materials. I wanted to keep ownership of the analysis and design decisions while using coding agents to make research, experimentation, implementation and revision more efficient.

Building a persistent project memory

Before receiving the full assignment instructions, I had already set up a GitHub repository around one idea: keep a living record of project decisions alongside the work, rather than relying on conversational history and model memory.

As the project grew, that record became essential infrastructure: a small set of Markdown (`.md`) files that Claude Code maintained as part of its regular workflow, while I directed how the system evolved. I introduced `RESOURCES.md` to track the growing set of learning resources and their intended purposes, and `QUOTES.md` as a reusable reference for the evidence I had flagged during my investigation. `DECISIONS.md` preserved choices, rationale and changes in direction.

The structure wasn't fixed from the start. An early `ARCHITECTURE.md` was retired once the architecture emerged through interconnected resource and meeting decisions; the work became assembling those decisions into a coherent strategy rather than maintaining a separate architecture document. This living documentation let both Claude and me revisit earlier reasoning, check for inconsistencies and keep continuity across sessions.

Building small tools to improve the work itself

I also used coding agents to build lightweight, purpose-built interfaces for different stages of the work. The first was an inbox-style reader for the original 75 source items. I wanted it to feel like working through real organizational communications, not a flattened pile of documents: I could follow the chronology, star significant messages and threads, archive others, annotate observations, and export those observations to bring back into working sessions. When I began developing the learning strategy, I had Claude consolidate my flagged evidence into `QUOTES.md`, which I used throughout design to check for blind spots and confirm that the concerns from the needs assessment were being addressed.

A project organizer became the evolving backbone of the project. It held the strategy and each resource as they developed, and its exported state helped reestablish context in later sessions and organize the overall design. For editorial review, another small artifact let me highlight passages, annotate specific concerns and export a targeted revision list, which I used across several review passes on the pre-read email and these documents.

I adopted a practical guideline for these tools: if one couldn't be built in one to three prompts, it probably wasn't worth building. That kept experimentation quick without letting the tools turn into separate development projects.

Separating collaboration from implementation

I used GPT and Claude Code in complementary roles. GPT was a strategic collaborator and independent critic: I used our discussions to analyze the evidence, challenge assumptions, compare alternatives, refine the strategy and develop implementation briefs. Its detailed, coordinated image-generation prompts for the resource thumbnails also reduced follow-up iteration and kept momentum. Claude Code worked directly in the repositories, building the resources and tools, checking source materials, maintaining the documentation, flagging inconsistencies and implementing revisions.

The roles weren't a rigid handoff, and I wasn't simply choosing between agent suggestions. Both models contributed ideas and challenged decisions, and I regularly brought in my own ideas, modifications and design calls. I decided what to adopt, change or set aside, and set the direction of the work.

Review, correction, and validation

The workflow followed a recurring pattern: develop, inspect, challenge, revise, lock. An early planning brief used approximately 600 Guides; Claude checked the source packet and found the correct figure, approximately 900. During slide development, GPT suggested wording that made the Plus credit sound conditional; Claude flagged the conflict with Finance and Product Operations guidance, and the requirement for both replacement and credit was preserved. Other reviews caught unsupported policy assumptions and attribution errors. Rather than fill those gaps with plausible answers, I kept the uncertainty visible and identified where stakeholder confirmation was needed. Those corrections showed why the review cycle mattered.

Automated checks helped catch implementation issues, but I treated reported test results as evidence of what had been checked, not proof of real-world usability or learning effectiveness. Independent user testing and screen-reader validation are still outstanding.

The harness didn't replace my judgment. It gave me more room to investigate, experiment, revisit decisions, and challenge the work before treating anything as finished.

04 Intentional tradeoffs

- **No simultaneous launch.** A full-population rollout would be faster to announce, but it would cost phone time and leave no way to learn what works first.
- **Lightweight resources over a course.** Each resource has one job and points to Beacon as the single authority, so nothing competes with it as a source of procedure.

- **Dependencies stay visible.** Where the case left something unresolved, the materials say who has to confirm it rather than resolving it for them.

05 What I'd validate

Readiness
Are Plus guidance, Orbit workflow and access, the Lead support route and fallback, practice delivery, and pilot scheduling confirmed by their owners?

Performance
On reviewed calls, are Guides recognizing, verifying, deciding, completing, communicating and documenting as intended?

Operational impact
Are complete recoveries increasing without unacceptable effects on handle time, wait and abandonment, or Lead and Specialist load?

A small pilot can surface problems and give directional evidence. It can't, on its own, prove the intervention caused an improvement, especially against a baseline whose eligibility was classified after the fact.

06 What remains uncertain

- The root cause of the gap, and how it splits across knowledge, tool access, policy clarity and workflow.
- How well Leads themselves know the WN7 path. The packet has no baseline, and what Leads asked for (cues, clips) may not be all they need.
- How to handle a pending Plus credit, the eligibility-review follow-up, and the fallback when a Lead is unavailable.
- Whether Sana can host the free-text practice natively, or only link to it.
- The materials have not yet demonstrated improved live-call performance; the pilot is designed to evaluate that.

07 What I'd do next in a real engagement

- Confirm policy and operational ownership, starting with Plus handling and Orbit access.
- Observe real Guide and Lead workflows rather than relying on meeting notes.
- Validate the Call Path, the practice model answers and the Lead materials with Quality and Specialist Care.
- Test usability with a few Guides and Leads, including screen-reader use.
- Agree the evaluation baseline and its limits with Quality.
- Pilot, gather evidence, and iterate before expanding.

Closing reflection

The request was for a video, but the evidence described a team being asked to carry out a new process under time pressure, with partial answers and uneven support. I tried to design for that reality: understand what each person has to do on the call or in the review, give them support at that point, and set up a way to see whether performance actually changes. Producing content was the smallest part of the job.